

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

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Finding Reasoning Paths :

- 1 . Persistent sadness -> Major Depressive Disorder (MDD) -> An hed onia .
- 2 . Hope lessness -> Major Depressive Disorder (MDD) -> An hed onia .
- 3 . Su ic idal ide ation -> Major Depressive Disorder (MDD) -> An hed onia .
- 4 . Chronic sadness -> Persistent Depressive Disorder (dyst h ym ia) -> An hed onia .
- 5 . Chronic sadness -> Adjustment Disorder with Depressed Mood -> An hed onia .

Reasoning Process :

- 1 . **Persistent sadness**: The patient reports feeling sad for as long as he can remember and describes his life as being "curse-cursed." This chronicity of sadness is a hallmark feature of persistent depressive disorders , such as dyst h ym ia , or major depressive disorder (MDD). However , the presence of additional symptoms (e .g ., hope lessness , suicidal ide ation , difficulty making decisions , energy , and concentration deficits) suggests that MDD is more likely than dyst h ym ia alone .
- 2 . **Hope lessness and depressive symptoms**: Hope lessness is a negative affect that is commonly seen in major depressive disorder . The patient’s feeling of being trapped and unable to improve his situation further supports this diagnosis .
- 3 . **Su ic idal ide ation**: Su ic idal ide ation is a severe symptom that is often associated with major depressive disorder . While it could also occur in other conditions (e .g ., bipolar disorder , psychosis), the absence of other manic or psychotic symptoms makes MDD the most likely diagnosis .
- 4 . **Difficulty making decisions and energy /c ognitive impairment**: These symptoms align with major depressive disorder , which often involves diminished motivation , executive functioning deficits , and fatigue .
- 5 . **Ex clusion of other conditions** :
 - **Sch izophren ia**: There is no evidence of halluc inations , del usions , or disorganized thinking to suggest psychosis .
 - **B ip olar disorder**: While suicidal ide ation can occur in bipolar disorder , there is no mention of manic or hyp oman ic episodes .
 - **Adjustment disorder**: The chronic nature of the symptoms (lasting as long as he can remember) makes this less likely .
 - **Other medical or neurological conditions**: The symptoms do not suggest a secondary cause (e .g ., medication -induced depression , vitamin deficiencies , or other medical conditions).
- 6 . **An hed onia**: An hed onia , the inability to experience pleasure , is a core feature of major depressive disorder . While persistent sadness alone is not an hed onia , the presence of other depressive symptoms strongly supports this diagnosis .

Conclusion :

The patient is most likely experiencing major depressive disorder with an hed onia . This conclusion is supported by the chronicity of sadness , hope lessness , suicidal ide ation , difficulty making decisions , energy deficits , and cognitive impairment , which are all consistent with MDD . An hed onia is a key symptom of MDD and aligns with the patient’s presentation .

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The patient is likely to show an hed onia .

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